

HOUSE No. 1386

The Commonwealth of Massachusetts

PRESENTED BY:

John J. Mahoney

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to ensure uniform and transparent reporting of medical debt data.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>John J. Mahoney</i>	<i>13th Worcester</i>	<i>1/17/2025</i>

HOUSE No. 1386

By Representative Mahoney of Worcester, a petition (accompanied by bill, House, No. 1386) of John J. Mahoney for legislation to establish an electronic system of public reporting for providers as a prerequisite condition for advancing overdue medical bills to debt collection agencies. Health Care Financing.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to ensure uniform and transparent reporting of medical debt data.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Section 7 of chapter 6D of the General Laws, as so appearing in the 2022
2 Official Edition, is hereby amended by striking subsection (d) in its entirety and inserting in
3 place thereof the following:-

4 (d) The commission shall consider proposals that achieve 1 or more of the following
5 goals: (i) to support safety-net provider and disproportionate share hospital participation in new
6 payment and health care payment and service delivery models; (ii) to support the successful
7 implementation of performance improvement plans by health care entities under subsection (c)
8 of section 10; (iii) to support cooperative efforts between representatives of employees and
9 management that are focused on controlling costs and improving the quality of care through
10 workforce engagement; (iv) to support the evaluation of mobile health and connected health
11 technologies to improve health outcomes among under-served patients with chronic diseases; (v)
12 to develop the capacity to safely and effectively treat chronic, common, and complex diseases in

13 rural and underserved areas and to monitor outcomes of those treatments; (vi) to reimburse the
14 center for health information analysis on all funds expended for the purposes of executing the
15 uniform medical debt reporting system established in section 25 of chapter 12C; and (vii) any
16 other goals as determined by the commission.

17 SECTION 2. Section 1 of chapter 12C of the General Laws, as so appearing, is hereby
18 amended by inserting after the definition of "Medical assistance program" the following
19 definition:-

20 “Medical debt”, any debt owed for goods or services provided by a medical facility, a
21 provider of health care or a provider of emergency medical services including the financing or an
22 extension of credit by a third party for the sole purpose of purchasing goods or services provided
23 by a medical facility, a provider of health care or a provider of emergency medical services.

24 And is further amended by inserting after the definition of "Self-insured group" the
25 following definition:-

26 “Significant medical debt”, any medical debt owed by an individual exceeding \$200.

27 SECTION 3. Said chapter 12C of the General Laws, as so appearing, is hereby further
28 amended by inserting after section 24 the following new section:-

29 Section 25. Uniform medical debt reporting system

30 (a) The center shall coordinate with the public health council, the boards of registration
31 for providers, the commission board, and the state finance and governance board to develop a
32 uniform and interoperable electronic system of public reporting for providers as a prerequisite
33 condition for advancing overdue medical bills to a debt collection agency.

(b) The uniform medical debt reporting system shall include information designed to advise on policy relating to medical debt. The uniform medical debt reporting system shall also ensure a comprehensive and transparent analysis of demographic data as it relates to medical debt including, but not limited to, rates of medical debt carried in the following demographics: (1) race; (2) sex, gender identity, and sexual orientation; (2) disability status; (3) criminal record; (4) health status; (5) family and individual income level; (6) education; (7) nation of origin; (8) region of residence in the commonwealth; (8) individual and family health insurance status; (8) veteran status; (9) age group; (10) chronic condition status; (11) education level; (12) primary language; and (13) times between procedures and reporting of debt to a collection agency.

(c) The purpose of the uniform medical debt reporting system is to reduce the adverse effects of medical debt and to protect patients in matters related to medical creditors, medical debt buyers, and medical debt collectors with respect to such debt. As such, the center shall collect and analyze data on all aspects related to the purposes of this section including, but not limited to: trends of medical debt assignment and collection per provider; rates of medical debt qualifying as “significant” as defined in section 1; the relative concentration of individual and family debt per person as compared to the total amount of medical debt in the commonwealth; any risks associated with masking medical debt data; the impact of medical debt data on public health and welfare; and dating relating to the rate at which those carrying medical debts successfully settle such debt.

(d) The center may centralize the uniform medical debt reporting system or create a central portal for public access to the medical debt data and information. The uniform medical debt reporting system shall be accessible to other state agencies and authorities including, but not

limited to, the commission, the secretary for the executive office of health and human services, the department of public health, and the state finance and governance board.

(e) The center shall promptly make available to the secretary of the executive office of health and human services all data pursuant to subsection (a) prior to a provider sending such debt information to a collection agency.

(f) The center shall coordinate with the commission, through its oversight and control of the Healthcare Payment Reform Fund, pursuant to section 7 of chapter 6D, to receive reimbursement funds for the purposes of executing the uniform medical debt reporting system as established in this section.

SECTION 4. Section 52 of chapter 93 of the General Laws, as so appearing, is hereby amended by inserting after clause (6) of subsection (a) the following two clauses:-

(7) Information concerning medical debt arising from the receipt of health care services.

(8) Medical debt which has:

(a) not yet been reported to the secretary of the executive office of health and human services pursuant to section 25(c) of chapter 12C;

(b) already been fully paid or settled; or

(c) existed for less than one year from the date of first acquisition.

SECTION 5. Said section 52 of said chapter 93, as so appearing, is hereby further amended by striking subsection (b) and inserting in place thereof the following subsection:-

75 (b) Except for the provisions in clause (8), the provisions of subsection (a) are not
76 applicable in the case of any consumer credit report to be used in connection with:

77 (1) a credit transaction involving, or which may reasonably be expected to involve, a
78 principal amount of fifty thousand dollars or more; or

79 (2) the underwriting of life insurance involving, or which may reasonably be expected to
80 involve, a face amount of fifty thousand dollars or more.